



Tort Law (9th edn)

Kirsty Horsey and Erika Rackley

p. 212 **8. Breach of duty: The standard of care**

Kirsty Horsey and Erika Rackley

<https://doi.org/10.1093/he/9780198925231.003.0008>

Published in print: 15 August 2025

Published online: August 2025

Abstract

This chapter focuses on breach of duty. Breach occurs where a defendant has fallen below the particular standard of care demanded by the law. This is largely an objective test and is determined by comparing the actions of the defendant to those imagined to be done in the same circumstances by the so-called 'reasonable man'. The questions to be answered are how the defendant ought to have behaved (what was the required standard of care) and how the defendant did behave (did they in fact fall below that standard).

Keywords: tort law, breach of duty, claim, negligence, standard of care

Problem question

Read this problem question carefully and keep it in mind while you are working through the chapter that follows. At the end of the chapter you will be able to apply what you have learnt to the problem question and advise the relevant parties.

Mpho and Gabriel have spent the afternoon on a charity walk alongside Little Amal who had been visiting Horsechester University.¹ Gabriel, who has just passed his driving test, offers Mpho a lift home. On the journey home Gabriel is distracted by fireworks celebrating the end of term. He loses control of the car and crashes into a lamp post. Mpho suffers minor cuts and bruises and is taken to hospital for a check-up.

At the hospital Aput is waiting to see a doctor. They have injured their right elbow. The doctor recommends surgery as ‘the best possible option’. It is well known that partial paralysis is a small but serious risk of the procedure. However, when Aput asks about any potential downsides, the doctor fails to answer. The operation goes ahead and is performed without negligence. Aput is now unable to bend their right arm.

Advise Mpho and Aput.

8.1 Introduction

Consider the following examples:

- *A cricketer hits a ball for six. It flies over the boundary fence hitting a passer-by on the head, causing serious injury.*
- *A learner driver mistakes the accelerator for the brake and hits a tree, injuring her instructor.*
- *An overworked junior doctor misreads the instructions on the back of a packet of drugs and injects a patient with 250 ml, rather than 25 ml as stated, paralysing the patient from the neck down.*

p. 213 There are three vital ingredients of the tort of negligence: duty of care, breach of duty and damage caused by the breach (factual and legal causation). This chapter focuses on the second of these requirements—breach of duty.

Breach occurs where the defendant falls below the particular standard of care demanded by the law. Establishing this involves making a value judgement on the defendant’s behaviour—was the defendant driving too fast given the particular road conditions? Did the doctor follow the correct procedure or administer the correct drug? Was appropriate safety equipment provided by the employer, and so on? The key issue to be addressed is how and where the courts set the acceptable standard or level of care owed towards one’s neighbours. There are two questions to be considered.

- (1) How the defendant *ought* to have behaved in the circumstances—what was the required standard of care in these circumstances? This is often described as a matter of law.
- (2) How the defendant *did* behave—did they (as a matter of fact) fall below the standard of care required?

As the majority of cases turn largely on the facts at hand—the speed of the car and whether it was appropriate for the time of day, location and so on—it is difficult (although not impossible) to make generalisations about how the defendant *ought* to have behaved, that is where the standard of care will be set. Moreover, the relation of many everyday negligence cases issues to duty and causation will be straightforward and so liability will

turn on breach, on what the defendant should and did (or did not) do. As such, despite its practical importance the *law* here is relatively limited. The most we can do is identify a few broad and rather open-ended propositions. Indeed, Tony Weir argues:

decisions on breach are not citable as *authorities*; they are merely *illustrations* of the application of the indubitable rule that if one is under a duty at common law such care must be taken as is called for in all the circumstances.²

However, this only goes so far. While clearly the determination of whether a breach has occurred depends on the particular facts of the case, it is nevertheless possible to identify some broad principles that the courts use when seeking to define the appropriate standard of care, which give some indication as to what the law requires us to do in order to avoid causing harm to others.

Pause for reflection

Some of the most familiar negligence cases—that is the ones most often reported in the media—involve ‘freak accidents’ during entertainment events or school trips.³ These have often been followed by calls for a public inquiry (to investigate the cause of the tragedy and make recommendations to prevent similar accidents from happening in the future) or a review ↵ of health and safety guidelines,⁴ as well as claims for compensation.⁵ In deciding these claims, the courts seek to strike a balance between ensuring safety and curtailing legitimate activities and/or commercial interests. The balance can be a tricky one:

Accidents happen, and sometimes they are what can be described as pure accidents in the sense that the victim cannot recover damages for the resulting injury because fault cannot be established. If the law were to set a higher standard of care than that which is reasonable ... the consequences would quickly become inhibited. There would be no fêtes, no maypole dancing and none of the activities that have come to be associated with the English village green for fear of what might conceivably go wrong. (Scott-Baker LJ, *Cole v Davies-Gilbert* at [36])

As you are reading the cases in this and other chapters, consider where you think the line should be drawn. What factors do you think might have pushed the courts one way or another?

8.2 A test of reasonableness

The first thing to note about the standard of care in negligence is that it is *not* a standard of perfection. The standard allows for *some* errors and mistakes. As Lord Atkin held in *Donoghue v Stevenson* [1932]: ‘You must take *reasonable care* to avoid acts or omissions which you can *reasonably foresee* would be likely to injure your

neighbour' (at 580, emphasis added). The key requirement is that the defendant behaves reasonably. Of course, this simply prompts the question: what is reasonable behaviour? Or, put another way, what standard of care did the circumstances require?

The courts answer this question by comparing what the defendant has done to the imagined actions of the so-called 'reasonable man' and asking what a reasonable man in the position of the defendant would have done in the circumstances. If the defendant has done something that the reasonable man *would not* have done (or has omitted to do something that the reasonable man *would* have done) then they will be in breach of their duty (*Hazell v British Transport Commission* [1958] at 171). In short,

[n]egligence is the omission to do something which the reasonable man, guided upon those considerations which ordinarily regulate the conduct of human affairs, would do, or doing something which a prudent and reasonable man would not do. (Alderson B, *Blyth v Birmingham Waterworks* (1856) at 784)

p. 215 ↵ So who is this apparent paragon of virtue against whom all others are to be judged? Over the years various descriptions have been attempted, often involving some mode of transport: the reasonable man is 'the man on the Clapham omnibus' (Greer LJ, *Hall v Brooklands Auto-Racing Club* [1933] at 224), 'the man who in the evening pushes his lawnmower in his shirtsleeves' (Lord Greer, *Hall v Brooklands Auto-Racing Club* [1933] at 224), a 'traveller on the London underground' (Lord Steyn, *McFarlane v Tayside Health Board* [1999] at 82) or, rather more grandly, he is 'the anthropomorphic conception of justice' (Lord Radcliffe, *Davis Contractors v Fareham Urban District Council* [1956] at 728).

Yet, while it is agreed that the reasonable man is neither all-seeing nor all-knowing and that, on occasions, he makes *reasonable* mistakes, there is less agreement (particularly among feminist legal scholars) about whether these qualities should be tied to the physical characteristics of a *man*. Moreover, it is clear that even if we adopt the more politically correct (if not less contentious) 'reasonable *person*' terminology, questions remain: what is her or his ethnicity, religion, class, sexual orientation, age, education, and so on?

Feminist critiques of the reasonable man

Feminist and other legal scholars have argued that despite claims to gender-neutrality, the standard of the reasonable man in fact embodies a male point of view and as such holds women to a standard devised without them in mind. This criticism goes beyond a dislike of the way the standard is expressed: 'his gender-specificity is not just a linguistic convention, whereby both sexes are denoted by reference to the masculine: the reasonable man is, in fact, male'.⁶

What does this mean? There are a number of aspects to this critique. One focuses on the fact that the notion of the 'reasonable man' is not based on statistical evidence of what most people actually do think and do. Rather, it is a conception conjured up by the judges themselves embodying a standard set and considered appropriate *by them*. Given the continuing under-representation of women, ethnic

minorities and other non-traditional groups within the judiciary, particularly in its upper echelons, it is likely that the 'reasonable man' reflects the particular perspectives, assumptions and (possibly) prejudices of a subset of society—that is, those (white, middle-class men) who make up the judiciary.

More controversially, it is argued that the standard of reasonableness not only assumes but *prioritises* the ability to adopt a position of detached objectivity from which to weigh up the costs and benefits of a particular course of action—an ability traditionally (and, perhaps, essentially) associated with men. Assuming this is true, then the very idea of applying 'a standard of reasonable care', it is argued, seems to embody a peculiarly *male* way of thinking and to exclude differences in the way women think and act. As such, the problem is not with the language of the reasonable 'man', but with the very notion of 'reasonableness'. On this view, it may well be 'dangerously misleading' to adopt the rebranding of the reasonable person—since it attributes a 'false universality to what is in fact a partial and loaded standard'.⁷

p. 216 ↩ The truth of the matter is, of course, that there is no such individual as the reasonable man (or indeed person). The reasonable man is a judicial construct—or shorthand—through which the judges seek to determine what was reasonable conduct in the circumstances of the particular case: the reasonable man is the personification of the judicial view of reasonableness. This does not mean, however, that the reasonable man is simply the personification of *the judges themselves*, or a decoy behind which they can and do pursue their own preferences and prejudices.⁸ Not least because there is a whole series of rules, developed through the case law, setting out the qualities and conduct—the 'standard of care'—expected of the defendant. It is to these that we now turn.

Test your understanding by trying this section's self-test questions. <<https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-self-test-questions-8-2>>.

8.3 An objective standard

The first thing to note is that the standard of 'what the reasonable man would do in the circumstances of the case' is objective. This means that the standard of care expected of the defendant is not dependent on, or skewed in favour of, certain characteristics and/or capabilities of the defendant; the personal idiosyncrasies of the defendant are largely (although, as we shall see, not completely) irrelevant (*Glasgow Corporation v Muir* [1943] at 457). Put simply (except for in a very few, limited, circumstances) the appropriate question is not 'what could *this particular* defendant have done?' but rather 'what level of care and skill did the activity the defendant was undertaking require?' What this means is that even though certain individuals may, due to certain inherent characteristics (e.g. experience and so on), be more or less able to take care the law imposes the same standard of care on everyone.⁹ The defendant cannot (usually) argue against, or raise as a defence in

response to, the imposition of liability on the grounds that they 'did their best' according to their education, experience, health and so on where their best falls below the standard of care expected of the reasonable man. This is clearly seen in the case of *Nettleship v Weston* [1971].

***Nettleship v Weston* [1971] 2 QB 691 (CA)**

Mrs Weston, the defendant, was learning to drive. The claimant had agreed to give her driving lessons. Mrs Weston was holding the steering wheel and controlling the pedals, while the claimant moved the gear lever and handbrake. Unfortunately, after turning a corner, the defendant failed to straighten the steering wheel and panicked. Despite her best efforts, and the fact that the car was only travelling at walking pace, the car mounted the pavement and hit a lamp post, breaking the claimant's knee cap. The defendant was convicted of driving without due care and attention.¹⁰ The claimant sued in negligence.

The majority of the Court of Appeal held that all drivers, *including those learning to drive*, are held to the same standard: 'The standard of care ... is measured objectively by the care to be expected of an experienced, skilled and careful driver' (at 702). The standard of care in negligence is not dependent on the particular defendant's characteristics and/or capacities. The fact that, as a learner, she was by definition incapable of meeting this standard was irrelevant: 'The learner driver may be doing his best, but his incompetent best is not good enough. He must drive in as good a manner as a driver of skill, experience and care' (at 699). Nevertheless, the claimant's damages were reduced by 50 per cent for contributory negligence as he was jointly controlling the car at the time of the accident.¹¹

The decision of the Court of Appeal in *Nettleship* is perhaps a little harsh. As a learner driver Mrs Weston was unable to reach the standard of care to which she was ultimately held. Moreover, the claimant was well aware of the defendant's limited driving capabilities—he was in the car *because* he had agreed to give her driving lessons. As Salmon LJ, in dissent, noted, a learner driver cannot owe an instructor a duty to drive with a degree of skill they both know is not possessed (at 705). However, in taking the opposite view and holding the defendant to an objective standard, the majority sought to avoid the 'endless confusion and injustice' that would follow should the standard of care be varied according to someone's knowledge of another's skill (at 700). By applying the same standard to all drivers, the decision in *Nettleship* ensures that victims of road accidents do not lose out simply because they were unlucky enough to be injured by someone who is learning to drive. The alternative is that the innocent victim of a learner driver who had driven as well as could be expected (albeit to a standard somewhat lower than that expected of an experienced driver) would be denied compensation.

Counterpoint

The ‘objective standard of care can be understood as the law’s attempt to strike a fair balance between the competing interests in freedom of action and personal security that we all share’.¹² But does the decision in *Nettleship* strike a ‘fair balance’ between the parties? Is it fair?

The reason we make liability in negligence conditional upon the defendant having failed to exercise reasonable care (rather than, for instance, making liability strict) is the belief that it is only where the defendant is at *fault* (and that fault causes the claimant’s injuries) that the law should require the defendant to pay compensation. Corrective justice does not require that we compensate *all* the harms we cause, only those that we cause through our fault or neglect. The problem with the decision in *Nettleship* is that it imposes liability without genuine fault.¹³

The decision in *Nettleship* is perhaps better understood as resting on a more pragmatic motivating factor—compulsory motor insurance. The majority judges ‘see to it’ that the defendant is

↪ liable so that the claimant can be compensated out of the insurance fund (at 700). In so doing, the Court of Appeal elevates the function of tort as a mechanism of compensation over that of achieving corrective justice, something that Lord Denning MR clearly recognises:

Thus we are, in this branch of the law, moving away from the concept: ‘No liability without fault’. We are beginning to apply the test: ‘On whom should the risk fall?’ Morally the learner driver is not at fault; but legally she is liable to be because she is insured and the risk should fall on her. (at 700)

Ultimately, of course, the financial loss falls neither on the claimant nor the defendant but on the defendant’s insurance company (and in turn other insurance policy holders). It is more accurate therefore to suggest that in *Nettleship* the fault principle is subordinated not only to ensure the claimant is compensated but also to the loss-shifting goals of distributive justice.

Despite appearances (underlined by *Nettleship*), the objective standard of care is not a standard of perfection. Take, by way of example, *Birch v Paulson* [2012] in which a driver was not liable for the serious injuries suffered by a drunk pedestrian who stepped out in front of her car. The Court of Appeal held that:

[while] it would have been very easy indeed for the defendant, as she approached this man on the kerb, either to have taken her foot off the accelerator or to have steered towards the centre of the road or both ... the legal test is not a question ... of perfection using hindsight. Of course it is not, and drivers are not required to give absolute guarantees of safety towards pedestrians. The yardstick is by reference to reasonable care ... there was nothing here to require the defendant as a reasonably careful driver to act in any way other than a way in which she did act given the situation in which she found herself at the time. (at [32])¹⁴

The willingness of the court to apply an objective standard of care, independent of the characteristics and capacities of the defendant, was taken to an extreme in *Roberts v Ramsbottom* [1980]. In this case, the defendant had (unknowingly) suffered a cerebral haemorrhage before starting to drive into town. Though he realised his consciousness was impaired, he was unaware that he was unfit to drive. As he was driving along, he suddenly began to feel much worse. He subsequently collided with a stationary van, narrowly avoided two men working in the road and knocked a cyclist onto the pavement, before ploughing into the claimant's car, seriously injuring the claimant and damaging the car. The defendant argued that from the moment of his stroke he was unable, through no fault of his own, to control his car properly or appreciate that he was unfit to drive. Neill J disagreed. Though the defendant was in no way to blame morally, he was nonetheless liable in the tort of negligence. He had fallen below the objective standard of the reasonable driver in relation to both in his collision with the claimant and also in continuing to drive despite feeling increasingly unwell and following his earlier collision with the parked van, even though he was, at the time, unable to

p. 219 appreciate their significance. It was acknowledged that if he had suffered a complete loss of consciousness he might have had a defence of 'automatism'.

In *Nettleship* and *Roberts v Ramsbottom* liability in negligence was imposed even though the defendant was not (or may not have been) genuinely at fault. However, the relevance of fault was reasserted in another dangerous driving case a few years later, *Mansfield v Weetabix Ltd* [1998], where the Court of Appeal held that the defendant was not liable for the damage caused to a shop (and shop owner's home) when the lorry he was driving crashed into it. Unknown to the defendant, he had a medical condition (malignant insulinoma) which starved his brain of glucose so that it was unable to function properly. The final accident, which formed the basis of the claim, was the culmination of a gradual lowering of his blood sugar level. However, crucially, the court held that at no time had the defendant been aware of his condition, despite the fact that over the course of 40 miles he had been involved in three distinct incidents, one of which involved the police. Leggatt LJ distinguished *Nettleship* on the ground that that case did not deal with the situation where the defendant was unaware of his disability (the learner driver knew that she was learning to drive) (at 1267) and *Roberts v Ramsbottom* on the basis that the defendant had 'continued to drive when he was unfit to do so and when he should have been aware of his unfitness' (at 1266–7). The defendant had not, therefore, fallen below (ie breached) the standard of care—that is of 'a reasonably competent driver unaware that he is, or may be, suffering from a condition that impairs his ability to drive' (at 1268). Leggatt LJ continued: 'To apply an objective standard in a way that did not take account of ... [the defendant's] condition would be to impose strict liability ... that is not the law' (at 1268).

Pause for reflection

It sometimes seems that recovery in tort can simply be a matter of luck.¹⁵ As Sarah Green and Jodi Garner point out, in both *Roberts v Ramsbottom* and *Mansfield v Weetabix* the claimant was injured by a driver whose ability to drive had been impaired by a medical condition. However, the specific nature of their conditions meant that while one claimant was successful, the other was not.¹⁶ Moreover, the

practical result, as acknowledged by Leggatt LJ, of *Mansfield v Weetabix* is that the claimants whose shop and home had been destroyed were left uncompensated and the defendant's pot of insurance money remained untouched:

the plaintiffs may well have been insured [e.g. household or business insurance]. Others in their position may be less fortunate. A change in the law is, however, a matter for Parliament. Meanwhile, since in my judgment ... [the defendant] was in no way to blame, he was not negligent. (at 1268)

Though they were aware of the harshness of the decision to the claimants, the Court of Appeal judges in *Mansfield v Weetabix*, unlike those in *Nettleship*, were unwilling to find the innocent lorry driver liable simply in order to compensate the claimants—who, after all, had (or should have had) their own *household* insurance. Responsibility for any change in the law, they argued, lay at the feet of Parliament (at 1268). Which approach do you prefer? Why? Think again about the arguments made earlier in relation to *Nettleship*.

p. 220 ↩ As *Mansfield v Weetabix* demonstrates the court have, on occasions, been willing to modify or vary the objective standard in relation to *some* illness:

Corr [v IBC Vehicles Ltd], *Mansfield v Weetabix* ... *Roberts [v Ramsbottom]* recognise that whether induced by severe acute and engaged symptoms of mental illness or an hypoglycaemic state, coronary thrombosis, or other physically irresistible impulse, on rare occasions evidence will show the alleged tortfeasor not in any meaningful sense the agent for the damage since autonomy and ability to reason and act was wholly eliminated. (Rafferty LJ, *Dunnage v Randall* at [19])

An example where this was *not* the case is *Dunnage v Randall* [2015]. The claimant, an undiagnosed paranoid schizophrenic, set himself on fire, injuring his nephew (who was attempting to prevent the accident). He was found to have fallen below the standard of care of a reasonable man. Rejecting the application of *Mansfield v Weetabix*, the Court of Appeal held that:

unless a defendant can establish that his condition entirely eliminates responsibility ... he remains vulnerable to liability if he does not meet the objective standard of care. It is the entirety of the elimination which drives this conclusion, and once that entirety is eroded or diminished, he is fixed with the standard. The evidence was that [the claimant's] responsibility came very close to complete elimination, but the experts stopped short of finding that it was complete. (Rafferty LJ at [114])¹⁷

Other occasions where the courts have been willing to modify or vary the objective standard to take into account *certain* characteristics of *some* defendants are where the defendant is a child or are professionals, professing to have a special skill or competence.

8.3.1 Children

It is accepted that the capacity of children to recognise the dangers of a particular activity, take care to avoid them and not hurt others varies according to the age of the child (unlike in criminal law there is no minimum age for liability in tort). In cases involving children, the standard of care applied is scaled according to age so that it becomes that which can be ‘objectively expected of a child of that age’ (*Orchard v Lee* [2009] at [9]). The question for the court is, in the words of Salmon LJ, ‘whether any ordinary child of 13½ [for example] can be expected to have done any more than this child did. I say “any ordinary child”. I do not mean a paragon of prudence; nor do I mean a scatter-brained child; but the ordinary girl of 13½’ (*Gough v Thorne* [1966] at 1387).

Orchard v Lee [2009] EWCA Civ 295 (CA)

Two 13-year-old boys were playing a game of tag in a courtyard and walkway designated for their year group during their school lunch break. During the course of the game the defendant was running backwards, taunting the other boy. Unfortunately he ran into the claimant, who was working as a lunchtime assistant supervisor at the school. The back of his head hit her cheek and, though the injury initially seemed insignificant, it later became quite serious. The trial judge dismissed

the supervisor’s claim finding that ‘it was a simple accident caused by “horseplay between two 13-year-old boys in and around an outside court yard ... boys doing what boys do”’ (at [2]).

The Court of Appeal agreed. A child will only be held liable in negligence if their conduct is careless to a very high degree or falls significantly outside the norm for a child of their age. In assessing whether this is the case, ‘the question is whether a reasonable 13-year-old boy, in the situation that [the defendant] was in, would have anticipated that some significant personal injury would result from his actions’ (Aikens LJ at [24]). As there was nothing to suggest that the defendant was playing tag ‘in a manner in which a 13-year-old boy would reasonably foresee there was likely to be injury beyond that normally occurring while a game of tag was in progress’, the defendant had not fallen below the standard of care required of him (Waller LJ at [12]).

Pause for reflection

As noted in the Australian case of *McHale v Watson* (1966), ‘Children, like everyone else, must accept as they go about in society the risks from which ordinary care on the part of others will not suffice to save them. One such risk is that boys of 12 may behave as boys of 12’ (at 216). Or, indeed, that 15-year-old schoolgirls will ‘play as somewhat irresponsible girls of 15’ (*Mullin v Richards* [1998] at 1312).

The ‘reasonable child’ standard invoked by the court effectively prevents a child-defendant’s action being measured against a standard of care that they are unable to reach. How—if at all—can this be reconciled with the Court of Appeal’s decision in *Nettleship*?

8.3.2 Professionals

Another area where the courts modify or vary the objective standard is where a person professes to have a special skill or competence. The law requires that when dealing with people in the context of a calling or profession they do so with an appropriate level of competence.

Nobody expects the passenger on the Clapham omnibus to have any skill as a surgeon, a lawyer, a pilot, or a plumber, unless he is one; but if he professes to be one, then the law requires him to show such a skill as any ordinary member of the profession or calling to which he belongs, or claims to belong, would display.¹⁸

In *Phillips v William Whiteley* [1938], for example, the court denied a claim for damages in respect of an infection the claimant developed after having her ears pierced in a jewellery store. Ear-piercing is not something which requires a surgical level of skill and so the claimant could expect only that her ears were pierced to the standard of a reasonably competent jeweller, which they had been in this case.¹⁹ In contrast, in *Wright v Troy Lucas (a firm) and George Ruzs* [2019], Eady J held an unqualified legal advisor to the standards of a professional lawyer:

I have no hesitation in finding that the defendants should be held to the duty and standard of care that they had chosen to assume when holding themselves out as competent to carry out legal services for the claimant in his clinical negligence litigation. (at [82])

Though the defendant had never described himself as a solicitor, he had held himself out as having ‘extensive experience’ of these types of claims and as being ‘as good as, if not better, than any solicitor or barrister’ (at [18]). He was ordered to pay over £260,000 in damages to compensate the claimant for what they were likely to have received had their original claim against the NHS been properly handled (on the advice of the defendant, the claimant had previously settled for £20,000 and incurred £73,000 in legal costs).

8.3.2.1 The *Bolam* test

Typically, a defendant cannot escape liability in negligence simply by arguing that they followed common practice: ‘Neglect of duty does not cease by repetition to be neglect of duty’ (*Bank of Montreal v Dominion Gresham Guarantee and Casualty Company* [1930] at 666). However, there is one important exception to this rule. The courts give wide latitude to professionals, acting in their professional capacity, to determine the standards by which they are to be judged. In cases where the defendant has a special skill or competence (i.e. a skill that the reasonable man ordinarily does not have) and the circumstances are such that they are required to exercise that skill or competence, the courts have developed a different approach. In such cases, the actions of the defendant are judged against those of the ordinary skilled man *professing to exercise that skill*—the so-called *Bolam* test.

***Bolam v Friern Hospital Management Committee* [1957] 1 WLR 582 (QBD)**

The claimant was a patient who had been given electro-convulsive therapy without being given a relaxant drug and without the appropriate physical restraints. In the course of the treatment, he fractured his hip, a possible consequence of the treatment about which he had not been warned. At the time, the medical profession held conflicting views on whether it was necessary to administer relaxant drugs before the procedure as a way of reducing the likelihood of injury and whether it was necessary to warn the patient of the risk of injury. In assessing the standard of care, the court held:

a man need not possess the highest expert skill ... it is sufficient if he exercises the ordinary skill of an ordinary competent man exercising that particular art ... he is not guilty of negligence if he has acted in accordance with a practice accepted as proper by a responsible body of medical men skilled in this particular art ... Putting it the other way around, a man is not negligent, if he is acting in accordance with such a practice, merely because there is a body of opinion that would take a contrary view. (McNair J at 586–7)

As such, the defendant was not in breach of his duty, as other responsible doctors would have acted in the same way.

p. 223 ↪ ***Bolam*** applies to *all* professionals exercising a special skill or competence. In *Moy v Pettman Smith* [2005], a case involving a claim in negligence against a barrister, Baroness Hale approved Lord Hobhouse's view in *Arthur JS Hall & Co v Simons* [2002] that:

the standard of care to be applied in negligence actions against an advocate is the same as that applicable to any other skilled professional who has to work in an environment where decisions and exercises of judgment have to be made in often difficult and time-constrained circumstances. It requires a [claimant] to show that the error was one which no reasonably competent member of the relevant profession would have made. (at [25])

However, a distinction is drawn between the defendant's own 'rank or status' and that of the 'post' they are occupying at the time of the alleged breach (*Wilsher v Essex Area Health Authority* [1987] (CA) at 751). The defendant's conduct is assessed according to the standard reasonably expected of a reasonably competent person occupying such a post. The law makes no allowances for inexperience or for the fact that everyone must, to a certain extent, gain practical experience 'on the job'. Thus in *Wilsher* a junior doctor was held to the standards of the more senior post they were occupying at the time, notwithstanding the fact that this was effectively part of their training: 'the standard is not just that of the averagely competent and well-informed junior houseman (or whatever the position of the doctor) but of such a person who fills a post in a unit offering a highly specialised service' (Mustill LJ at 751).²⁰ *Wilsher* was applied in *FB (Suing by her Mother and*

Litigation Friend, WAC) v Princess Alexandra Hospital NHS Trust [2017]. The defendant, a Senior House Officer (SHO), was found to have fallen below the standard of care of a reasonably competent SHO in her treatment of a young child:

Dr Rushd [the defendant] in the present case must be judged by the standard of a reasonably competent SHO in an accident and emergency department. The fact that Dr Rushd was aged 25 and ‘relatively inexperienced’ ... does not diminish the required standard of skill and care. On the other hand, the fact that she had spent six months in a paediatric department does not elevate the required standard. Other SHOs in A&E departments will have different backgrounds and experience, but they are all judged by the same standard ... Thus in professional negligence, as in the general law of negligence, the standard of care which the law requires is an imperfect compromise. It achieves a balance between the interests of society and fairness to the individual practitioner. (at [63], [60])

Counterpoint

In practice, the **Bolam** test means that the courts generally defer to the standards of a particular profession as to what is considered appropriate behaviour. This is important. As Margaret Brazier and José Miola have argued, ‘judges are not qualified to make professional judgements on the practices of other learned professions ... A professional should not be penalised, and held to be incompetent, just because a judge fancies “playing” at being architect, solicitor or doctor’.²¹

p. 224

↪ A balance needs to be struck between respect for expert opinions and showing them undue deference.²² It is argued that the courts have been reluctant to impose liability on members of the medical profession. Traditional justifications for this include: the importance of the doctor’s reputation as a member of a respectable and responsible profession, a reluctance to interfere in a specialised branch of knowledge, the view that doctors are motivated by altruistic reasons and that it would therefore be unfair to penalise them for every error and, finally, a fear of increased claims and defensive medical practices.²³

In her exploration of the gendered dimensions of the **Bolam** principle and its conflation of *accepted* with *acceptable* practice, Sally Sheldon suggests a number of alternative justifications pointing in the opposite direction, that is for imposing liability. These include the need to compensate the victims of medical misadventures, the potential of negligence claims to discourage bad practice and reinforce the accountability of doctors (especially given their high status and remuneration), the current failure to recognise the vulnerability of the patient in the doctor/patient relationship and, crucially, the existence of liability insurance.²⁴

Which of these views do you find the most persuasive?

While *Bolam* established that a doctor will not have fallen below the standard of care expected of a reasonable doctor, if they have acted in accordance with a respectable body of the medical profession (even if it is a minority viewpoint), it was qualified by a ‘gloss’ applied by the House of Lords in *Bolitho v City and Hackney Health Authority* [1998]. In this case, a young boy suffered brain damage and later died of a cardiac arrest following respiratory failure while in hospital. Two serious incidents of respiratory difficulties had preceded his death, during which nurses had called for the doctor, who never arrived. As a result, the young boy had not been intubated (which the claimant argued would have prevented his death). The doctor argued that, even if she had attended, she would not have intubated the young boy and that, as a result, her failure to attend had not caused his death. Ultimately, though the majority of witnesses agreed that the boy should have been intubated, the House of Lords was persuaded that—as a reasonable body of opinion would have acted as the defendant did—she had not fallen below the standard of care required of her. However, the court went on to stress that the common practices of a profession should not go completely unchecked:

In the vast majority of cases the fact that distinguished experts in the field are of a particular opinion will demonstrate the reasonableness of that opinion ... But if, in a rare case, it can be demonstrated that the professional opinion is not capable of withstanding logical analysis, the judge is entitled to hold that the body of opinion is not reasonable or responsible. (Lord Browne-Wilkinson at 243)

p. 225 ↪ The upshot of *Bolitho* is that ‘doctor knows best’ *only* ‘if he [sic] acts reasonably and logically and gets his facts right’.²⁵ If a particular practice, even if widely accepted within a profession, is logically insupportable, a defendant cannot escape liability simply by showing that others would have acted as they did.²⁶ What this means is that the *Bolam* test is now better understood, to use Rachael Mulheron’s formulation, as a two-stage test:

- (1) Has the doctor acted in accordance with a practice accepted as proper by a respectable body of medical opinion?
- (2) If yes, is the practice itself ‘reasonable’ and ‘logical’?²⁷

However, crucially, Mulheron notes, the courts have not added a third stage in which they choose between two reasonable and logical schools of thought:

[I]t is *not* for the court to venture into a consideration of two contrary bodies of opinion and to decide the case on the basis of which, of the patient’s and the doctor’s expert medical opinion, it prefers ... Once *Bolam* applies, the fact of differences in expert opinion cannot lead to a rejection of *Bolam* evidence.²⁸

The assumption that ‘doctors know best’ (albeit qualified by the *Bolitho* gloss) has been especially controversial in relation to warning patients of the risks of treatment and the treatment of medically incapacitated adults and mature minors.²⁹ The former now appears to have been resolved following the recent case of *Montgomery v Lanarkshire Health Board* [2015] in which the Supreme Court rejected the application of the *Bolam* test in cases involving the disclosure of risks in a medical context.

Until *Montgomery*, the leading case on the duty of doctors to inform their patients of the risks of their treatment was the 30-year-old decision in *Sidaway v Bethlem Royal Hospital* [1985]. In *Sidaway*, the claimant had been paralysed while undergoing an operation on her back. There was no evidence that the operation had been performed negligently. However, the claimant argued that she should have been told of the known (albeit very small) risk of paralysis accompanying the procedure and that, had she been told, she would not have had the operation. The majority of the House of Lords, applying *Bolam*, held that the doctor had not breached his duty of care in failing to inform her of the risks associated with her treatment as other reasonable doctors would have acted in the same way. There was one caveat however. In cases where the risk of ‘grave adverse consequences’ was ‘substantial’—the assessment of which was to be left to the doctor—‘in the absence of some cogent clinical reason why the patient should not be informed, a doctor, recognising and respecting his patient’s right of decision, could hardly fail to appreciate the necessity for an appropriate warning’ (Lord Bridge, *Sidaway* at 24).

- p. 226 ↵ The majority view has been the subject of extensive and scathing criticism, much of which has focused on Lord Scarman’s dissenting opinion, in which he argued that the courts ‘cannot stand idly by if the profession, by an excess of paternalism, denies its patients real choice. In a word, the law will not allow the medical profession to play God’ (at 1028). Indeed, as Lords Reed and Kerr noted in *Montgomery*, it would be ‘wrong to regard *Sidaway* as an unqualified endorsement of the application of the *Bolam* test to the giving of advice about treatment’ (at [57]). Over the years, the unsatisfactory nature of the majority’s analysis in *Sidaway* has meant that the courts in England and Wales have tacitly departed from it (in favour of Lord Woolf MR’s test in *Pearce v United Bristol Healthcare NHS Trust* [1999]): a position that ‘was effectively endorsed, particularly by Lord Steyn, in the causation case *Chester v Afshar* [2005]³⁰ and was unequivocally confirmed in *Montgomery*.

***Montgomery v Lanarkshire Health Board* [2015] UKSC 11 (SC)**

In 1999, the claimant gave birth to a baby boy. As a result of complications during the birth, during which the baby was starved of oxygen, the baby was born with significant disabilities. The claimant sued in negligence (on behalf of her son). The claimant was a relatively small woman and diabetic. Women with diabetes are more likely to have large babies and there is a 9–10 per cent risk of shoulder dystocia during vaginal delivery (the baby’s shoulders being too wide to pass through the mother’s pelvis). The claimant was told that she was having a larger than usual baby; however she was not told about the risks of her experiencing mechanical problems during labour. Her doctor accepted that there was a high risk of shoulder dystocia, but despite this, she said that her practice was not to spend a lot of time, or indeed any time at all, on it ‘because, in her estimation, the risk of a grave problem for the baby resulting from shoulder dystocia was very small ... and that if the condition was mentioned, “most women will actually say, ‘I’d rather have a caesarean section’ ... and it’s not in the maternal interests for women to have caesarean sections”’ (at [13]).

The primary question for the court was whether, in failing to warn the claimant of the risk, the doctor was in breach of her duty of care towards the claimant. (The lower courts had found the doctor's actions to accord with a responsible body of medical opinion (as required by *Bolam*) and—surprisingly—not to fall within the small group of cases involving 'a substantial risk of grave adverse consequences' where the court may substitute its own view.)

The Supreme Court unanimously found in favour of the claimant. It held that patients have a right to make their own decisions and to be given sufficient information to do so. Doctors, therefore, have a corresponding duty to take reasonable care to ensure that a patient is aware of material risks inherent in treatment, and of any alternatives:

Since *Sidaway*, however, it has become increasingly clear that the paradigm of the doctor–patient relationship implicit in the speeches in that case has ceased to reflect the

↪ reality and complexity of the way in which healthcare services are provided, or the way in which the providers and recipients of such services view their relationship ... patients are now widely regarded as persons holding rights, rather than as the passive recipients of the care of the medical profession ... In the law of negligence, this approach entails a duty on the part of doctors to take reasonable care to ensure that a patient is aware of material risks of injury that are inherent in treatment ...

An adult person of sound mind is entitled to decide which, if any, of the available forms of treatment to undergo, and her consent must be obtained before treatment interfering with her bodily integrity is undertaken. The doctor is therefore under a duty to take reasonable care to ensure that the patient is aware of any material risks involved in any recommended treatment, and of any reasonable alternative or variant treatments. The test of materiality is whether, in the circumstances of the particular case, a reasonable person in the patient's position would be likely to attach significance to the risk, or the doctor is or should reasonably be aware that the particular patient would be likely to attach significance to it. (Lords Reed and Kerr at [75], [82], [87])

The Justices made clear that significance of a risk is fact-sensitive and cannot be reduced to percentages, and that (in line with guidance issued by the General Medical Council) in order to advise, the doctor must engage in dialogue with her patient (at [89]).

There is one important caveat (as well as the expected exceptions related to emergency treatment) to the doctor's duty to inform in the form of a 'therapeutic exception'. This allows a doctor to withhold information where 'he reasonably considers that its disclosure would be seriously detrimental to the patient's health' (at [88]). The court anticipates that use of this caveat will be limited: 'it is not intended to subvert that principle by enabling the doctor to prevent the patient from making an informed choice where she is liable to make a choice which the doctor considers to be contrary to her best interests' (at [91]).

p. 227

A secondary question for the court related to causation. The lower courts held that the claimant had not established on the facts that she would have elected to have a caesarean section, had she been properly informed. Given the evidence of the doctor herself that ‘if you were to mention shoulder dystocia to every [diabetic] patient, if you were to mention to any mother who faces labour that there is a very small risk of the baby dying in labour, then everyone would ask for a caesarean section’, and the claimant’s own statements, it is unsurprising that the Supreme Court concluded that the claimant would have chosen a caesarean section had it been offered to her.³¹

p. 228

Pause for reflection

Montgomery is also—particularly in Lady Hale’s ‘footnote’ to the main judgment—a timely and important recognition of patient—and in particular women’s—autonomy. It offers a ‘robust’ defence of women’s choice in the context of pregnancy and childbirth.³²

... it is not possible to consider a particular medical procedure in isolation from its alternatives. Most decisions about medical care are not simple yes/no answers. There are choices to be made, arguments for and against each of the options to be considered, and sufficient information must be given so that this can be done ... Pregnancy is a particularly powerful illustration [of this] ... That is not necessarily to say that the doctors have to volunteer the pros and cons of each option in every case, but they clearly should do so in any case where either the mother or the child is at heightened risk from a vaginal delivery. In this day and age, we are not only concerned about risks to the baby. We are equally, if not more, concerned about risks to the mother. And those include the risks associated with giving birth, as well as any after-effects. One of the problems in this case was that for too long the focus was on the risks to the baby, without also taking into account what the mother might face in the process of giving birth. (at [109]–[111])

Lady Hale continued,

We do not have a full transcript of the evidence, but in the extracts we do have Dr McLellan [the defendant doctor] referred to explaining to a mother who requested a caesarean section ‘why it may not be in the mother’s best interest’ and later expressed the view that ‘it’s not in the maternal interests for women to have caesarean sections’. Whatever Dr McLellan may have had in mind, this does not look like a purely medical judgment. It looks like a judgment that vaginal delivery is in some way morally preferable to a caesarean section: so much so that it justifies depriving the pregnant woman of the information needed for her to make a free choice in the matter. Giving birth vaginally is indeed a unique and wonderful experience, but it has not been suggested that it inevitably leads to a closer and better relationship between mother and child than does a caesarean section.

In any event, once the argument departs from purely medical considerations and involves value judgments of this sort, it becomes clear ... that the *Bolam* test, of conduct supported by a responsible body of medical opinion, becomes quite inapposite. A patient is entitled to take into account her own values, her own assessment of the comparative merits of giving birth in the ‘natural’ and traditional way and of giving birth by caesarean section, whatever medical opinion may say, alongside the medical evaluation of the risks to herself and her baby. She may place great value on giving birth in the natural way and be prepared to take the risks to herself and her baby which this entails. The medical profession must respect her choice, unless she lacks the legal capacity to decide (*St George’s Healthcare NHS Trust v S* [1999]). There is no good reason why the same should not apply in reverse, if she is prepared to forgo the joys of natural childbirth in order to avoid some not insignificant risks to herself or her baby. She cannot force her doctor to offer treatment which he or she considers futile or inappropriate. But she is at least entitled to the information which will enable her to take a proper part in that decision ... Gone are the days when it was thought that, on becoming pregnant, a woman lost, not only her capacity, but also her right to act as a genuinely autonomous human being. (at [114]–[116])³³

Lady Hale retired from the Supreme Court in January 2020. However, her comments in *Montgomery* are indicative of what we came to expect—and welcome—from the first woman Justice on the Supreme Court during her time on the court.³⁴ Why do you think she decided to add a ‘footnote’ in this case? Do you think it is important that she did so?

Counterpoint

And yet, while *Montgomery* calls for the opening up of lines of communication between medical professionals and patients, as Kirsty Keywood notes, the effectiveness of the case in promoting and protecting patient choice is—at least in part—dependent on resources being available to support their choices.³⁵ Could the decision in *Montgomery* itself work against this?

After all, one consequence of the decision in *Montgomery* is that a substantial sum of the Lanarkshire NHS Board's budget has been taken out of the collective pot. Maternity claims already represent the highest value, and the second highest number, of all NHS claims (though these represent just 0.1 per cent of all births).³⁶ However, while Lords Reed and Kerr acknowledged that the decision *could* lead to an increase in defensive practices, as well as in claims from women claiming that they had not been adequately advised, they did not expect this to be the case:

[A]n approach to the law which, instead of treating patients as placing themselves in the hands of their doctors (and then being prone to sue their doctors in the event of a disappointing outcome), treats them so far as possible as adults who are capable of understanding that medical treatment is uncertain of success and may involve risks, accepting responsibility for the taking of risks affecting their own lives, and living with the consequences of their choices ... in so far as the law contributes to the incidence of litigation, an approach which results in patients being aware that the outcome of treatment is uncertain and potentially dangerous, and in their taking responsibility for the ultimate choice to undergo that treatment, may be less likely to encourage recriminations and litigation, in the event of an adverse outcome, than an approach which requires patients to rely on their doctors to determine whether a risk inherent in a particular form of treatment should be incurred. (at [81], [93])

Do you agree?³⁷

p. 230 ↩ Without doubt *Montgomery* is a landmark decision.³⁸ “‘Informed consent’ has arrived’ and is now ‘firmly part of English law’ (Lady Hale at [107]).³⁹ Doctors now have ‘a duty to take reasonable care to ensure that the patient is aware of any material risks involved in any recommended treatment, and of any reasonable alternative or variant treatments’ (at [87]). In *Duce v Worcestershire Acute Hospitals NHS Trust* [2018], Hamblen LJ suggested the assessment of the materiality of the risk involved a two-stage test (paragraph numbers refer to *Montgomery*):

- (1) What risks associated with an operation were or should have been known to the medical professional in question. That is a matter falling within the expertise of medical professionals [83].
- (2) Whether the patient should have been told about such risks by reference to whether they were material. That is a matter for the Court to determine [83]. This issue is not therefore the subject of the *Bolam* test and not something that can be determined by reference to expert evidence alone [84]–[85]. (*Duce* at [33])

So when might a risk be ‘material’? And, more importantly, how does a doctor determine what is material to a specific patient? The test of materiality, as set out in *Montgomery*, is ‘whether, in the circumstances of the particular case, a reasonable person in the patient’s position would be likely to attach significance to the risk, or the doctor is or should reasonably be aware that the particular patient would be likely to attach significance to it’ (at [87]). The test is both subjective and objective. However, what is less clear is the extent to which subjective factors relating to the patient are relevant:

[T]he greater degree of subjectivity inserted into the assessment the further one departs from the standard of the reasonable patient. Some characteristics of a patient are obvious: In particular that person’s actual medical condition which would include its severity. Other personal factors may be less self-evident: such as the patient’s tolerance for or stoicism towards pain, or the ability to manage pain. Other factors might be quite remote from the medical or physiological condition of the patient, such as the patient’s need to return to work, or the fact that the patient has suffered a recent event in his/her life (such as a bereavement or a divorce) which renders that person unusually fragile and (say) unwilling to take chances at that particular time. (*Green J, Thefaut v Johnston* [2017] at [55])

The test was applied by the High Court in *A v East Kent Hospitals University NHS Foundation Trust* [2015] (which was heard just five days after the judgment in *Montgomery*). The claimant argued that the defendant had negligently failed to warn her that the foetus she was carrying might be suffering from a chromosomal abnormality. It was common ground that were the risk of such an abnormality found to be ‘material’ then it ought to have been raised with the claimant. Finding for the defendant, the judge held that while a risk ‘somewhere in the region of 1–3 per cent’ would, in this context, be considered ‘material’ (and, as such, should be raised with the claimant), ‘the decision in *Montgomery* ... is not authority for the proposition that medical practitioners need to warn about risks which are theoretical’ or ‘background’, as was the case here (*Dingeman J* at [90], [101]).⁴⁰

p. 231 ↪ But what of the availability of ‘any reasonable alternative or variant treatments’. Lords Kerr and Reed left open the question as to when an alternative treatment would be considered ‘reasonable’ and, as such, raised with the patient. This was addressed by the Supreme Court in *McCulloch v Forth Valley Health Board* [2023].

McCulloch v Forth Valley Health Board [2023] UKSC 26 (SC)

Mr McCulloch was admitted to hospital with severe chest pains. He was diagnosed with pericarditis (inflammation of the sac surrounding the heart). Following treatment, his condition improved and he was discharged from hospital. Two days later, he was readmitted complaining of chest pain. He was seen by a consultant who discussed with him the possibility of pericardiocentesis (a procedure to drain the fluid) but, as his condition appeared to be improving, she did not think this was necessary at the time. The consultant did not, however, discuss her decision not to prescribe Mr McCulloch non-steroidal anti-inflammatory drugs (NSAIDs) as an alternative to the procedure. A few days later, he was discharged from hospital. Sadly, Mr McCulloch died the next day following a cardiac arrest. The claimant (Mr McCulloch's widow who was bringing the claim along with other relatives) argued that Mr McCulloch ought to have been prescribed NSAIDs and that had the consultant done so, Mr McCulloch would have taken them and, on the balance of probabilities, would not have suffered a cardiac arrest.

The question for the court was 'what legal test should be applied to the assessment as to whether an alternative treatment is reasonable and requires to be discussed with the patient' (at [3]). And, more specifically,

did the doctor in this case fall below the required standard of reasonable care by failing to make a patient aware of an alternative treatment in a situation where the doctor's opinion was that the alternative treatment was not reasonable and that opinion was supported by a responsible body of medical opinion? (at [3])

Finding for the defendant, the court held that 'the correct legal test to be applied to the question of what constitutes a reasonable alternative treatment is the professional practice test found in ... *Bolam*' (at [56]). Lords Hamblen and Burrows continued:

On the facts of this case, therefore, as Dr Labinjoh [the consultant] took the view that prescribing NSAIDs was not a reasonable alternative treatment because ... [Mr McCulloch] had no relevant pain and there was no clear diagnosis of pericarditis and, because that view was supported by a responsible body of medical opinion ... there was no breach of the duty of care to inform required by *Montgomery* ... [and] no basis for going behind [the] decision [of the lower courts who had] reached on the evidence that Dr Labinjoh was not negligent. (at [56])

They further stated that:

A hypothetical example may help to explain, in more detail, how we regard the law as working. A doctor will first seek to provide a diagnosis (which may initially be a provisional diagnosis) having, for example, examined the patient, conducted tests, and having had discussions with the patient. Let us then say that, in respect of that diagnosis, there are ten possible treatment options and that there is a responsible body of medical opinion that would regard each of the ten as possible treatment options. Let us then say that the doctor, exercising his or her clinical judgment, and supported by a responsible body of medical opinion, decides that only four of them are reasonable. The doctor is not negligent by failing to inform the patient about the other six even though they are possible alternative treatments. The narrowing down from possible alternative treatments to reasonable alternative treatments is an exercise of clinical judgment to which the professional practice test should be applied. The duty of reasonable care would then require the doctor to inform the patient not only of the treatment option that the doctor is recommending but also of the other three reasonable alternative treatment options (plus no treatment if that is a reasonable alternative option) indicating their respective advantages and disadvantages and the material risks involved in such treatment options.

The identification of which treatments are reasonable alternatives (ie clinically appropriate) is as much a matter falling within medical expertise and professional judgment, and hence governed by the professional practice test, as the identification of risks associated with any treatment. Indeed, they are closely linked. The risk of any given treatment will be a significant part of any analysis of alternative treatment options. The identification of reasonable alternative treatments (ie clinically appropriate treatments) should therefore be treated in the same way as the identification of risk in *Duce*. It is only once the reasonable alternative treatment options have been identified that the second stage advisory role arises. That is, the doctor is required at the second stage to inform the patient of the reasonable alternative treatments and of the material risks of such alternative treatments. (at [57], [64])

Though welcomed by NHS Resolution for applying ‘sound common sense’,⁴¹ as Louise Austin has noted, the upshot of *McCulloch* is the creation of ‘two tests of disclosure dependent upon whether the information being disclosed relates to risks or alternative treatments’:

Montgomery is widely regarded as a landmark decision in informed consent because of its redefining of ‘materiality’ of risk away from the *Bolam* reasonable doctor standard in favour of an approach focused on what information the patient is likely to find significant. The courts have not adopted the same patient-oriented approach in the context of reasonable alternative treatments culminating in the Supreme Court’s decision to apply *Bolam* in *McCulloch*.⁴²

p. 233 Inevitably, perhaps, many legal arguments have been made, and much judicial and academic ink⁴³ has been spilt since *Montgomery* as doctors, lawyers and their clients attempt to work out its impact on clinical practice.⁴⁴ What is clear is that *Bolam* is far from dead. It continues to inform both stages of the *Montgomery* test (see e.g. *Grimstone v Epsom and St Helier University Hospitals NHS Trust* [2015]; *Bayley v George Eliot Hospital* [2017]; *Duce*; *Diamond v Royal Devon and Exeter NHS Foundation Trust* [2019]; *Brady v Southend University Hospital NHS Foundation Trust* [2020]). Nor has *Montgomery* been confined to recent cases. In *CNZ v Royal Bath Hospitals NHS Foundation Trust* [2023], for example, it was applied in respect of obstetric care received by the claimant's mother in 1996. Neither has it been confined to consent cases (as was originally thought). Its principles have been applied in cases where treatment in general is in question (see e.g. *Webster v Burton Hospitals NHS Foundation Trust* [2017]), to the duty to inform a patient of the need for follow-up treatment (*Gallardo v Imperial College Healthcare Trust* [2017]) and outside a medical context in *O'Hare v Coutts & Co* [2016] in which the defendant, a private bank, was found to owe a duty to take reasonable care to warn its customers of risks relating to suggested investments (although, on the facts, it had not breached that duty).

Test your understanding by trying this section's self-test questions <<https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-self-test-questions-8-3>>.

8.4 Setting the standard of care

Having determined that the courts generally apply a common standard, we now need to consider how the courts go about *setting* that standard. While the courts do not (usually) take into account the characteristics of the individual defendant, they do take into account the circumstances of the situation in which the accident or injury occurred. The standard of care does not exist in the abstract: 'The law in all cases exacts a degree of care commensurate with the risk' (Lord Macmillan, *Read v Lyons Co Ltd* [1947] at 173).

So, if we ask what is a reasonable speed to drive, the answer will vary—the reasonable car driver will drive at different speeds at different times and according to different circumstances.⁴⁵ Some examples may help here. All things being equal, it is reasonable to drive at 70 mph on a clear motorway but not on residential streets. It will be reasonable to drive faster in clear weather conditions than when it is pouring with rain or foggy. Why is this? The answer is straightforward. The risks posed by driving at 70 mph in a residential street are higher than those on a clear motorway. Similarly, accidents are more likely to occur in adverse weather conditions. The reasonable driver may be expected to take more care when driving near a school, where potential injuries may not only be more likely (e.g. a child running out from behind a parked car) but more severe (a child is more likely to be more seriously injured than an adult if hit by a car) than if, say, they have an injured friend in the back of their car who they are rushing to hospital (where the social utility of their act may outweigh the risks it creates).

Thus, in *Wooldridge v Sumner* [1963], for example, a photographer who was injured while working at the National Horse Show was unable to recover for the serious injuries he suffered when one of the riders he was photographing had come round the corner of the arena at excessive speed causing the horse to become temporarily out of control. Rejecting his claim, the Court of Appeal held that:

[t]he law of negligence has always recognised that the standard of care which a reasonable man will exercise depends upon the conditions under which the decision to avoid the act or omission relied upon as negligence has to be taken. (Diplock LJ at 67)

In the absence of recklessness or an intention to harm the claimant, the horse rider was free to concentrate on winning the competition.

When seeking to define the standard of care required of the defendant—that is, what the defendant *ought* to have done or what is *reasonable* in the circumstances of the case—the courts take a number of factors into account: probability or risk of the injury; the seriousness of the injury; the cost of taking precautions; and the social value of the activity:

the question of what amounts to ‘such care as is in all the circumstances of the case is reasonable’ depends upon assessing ... not only the likelihood that someone may be injured and the seriousness of the injury which may occur, but also the social value of the activity which gives rise to the risk and the cost of preventative measure. These factors have to be balanced against each other. (Lord Hoffmann, *Tomlinson v Congleton Borough Council* [2003] at [34])

It is, in essence, a balancing act where no one factor is viewed in isolation from the others. In each case, the courts weigh up the likelihood of injury and the seriousness of harm against the cost of taking precautions and the social value of the activity before coming to a decision as to what was reasonable in all the circumstances of the case—that is, what the reasonable man would have done.

Pause for reflection

The process through which the courts distinguish between acceptable and unacceptable carelessness was reduced to a quasi-mathematical formula by a US judge named Learned Hand (in *United States v Carroll Towing Co* [1947]). The so-called ‘Learned Hand test’ holds that where the private and social costs⁴⁶ of taking precautions (B) is less than the probability (P) multiplied by the likelihood of injury (L), the defendant will be held to have fallen below the standard of care of the reasonable person if they fail to take such precautions. Conversely, where the costs of taking precautions (B) are greater than the probability (P) multiplied by the seriousness of the injury (L) occurring, it would be unreasonable to expect the defendant to take such precautions.

Of course, in reality few, if any, social interactions can be reduced to this formula. And while the test remains a helpful *illustration* of the sort of balancing exercises the courts are undertaking when setting the appropriate standard of care in all the circumstances of the case, its purpose is essentially economic: 'If it can be shown that expenditure of £X on avoiding or minimising the risk of an accident will prevent accident costs of £X + Y, then it is clearly desirable that £X should be spent. On the other hand, it is said, there is no point in spending £X to prevent accident costs which are less than £X.'⁴⁷ This reduction of value of life and limb to a mathematical formula is described by Joanne Conaghan and Wade Mansell as 'ideologically objectionable in its shallow and impoverished view of human activity'.⁴⁸ Do you agree? For a real-life example consider the US case of *Grimshaw v Ford Motor Co* [1981], discussed in **section 12.2.1.1**.

8.4.1 Probability that the injury will occur

One of the first factors the courts will take into account is likelihood of the injury occurring. The general rule is the more likely—or foreseeable—the outcome, the greater the possibility that the courts will find the defendants liable for failing to take steps to avoid it.

***Bolton v Stone* [1951] AC 850 (HL)**

The claimant was hit on the head outside her house by a cricket ball hit by a player from an adjacent cricket pitch. The hit was substantial, although possibly not exceptional. The ball had travelled some 100 yards (approx 91 m) and over a seven-foot fence (which, due to the slope of the pitch, was in effect 17 feet high). The claimant sued in negligence and nuisance.

The House of Lords, rejecting both claims,⁴⁹ held that although there was evidence that over a period of years balls had been hit out of the ground, this was, in fact, very rare. It had happened six times in 30 years. Thus, although the possibility of a ball being hit out of the ground was foreseeable, this was not sufficient to establish negligence since the chances of the ball hitting someone were so low that a reasonable person would not have taken precautions to stop it happening.

The standard of care in the law of negligence is the standard of an ordinarily careful man, but in my opinion an ordinarily careful man does not take precautions against every foreseeable risk ... He takes precautions against risks which are reasonably likely to happen ... there are many footpaths and highways adjacent to cricket grounds and golf courses on to which cricket and golf balls are occasionally driven, but such risks are habitually treated both by the owners and committees of such cricket and golf courses and by the pedestrians who use the adjacent footpaths and highways as negligible and it is not, in my opinion, actionable negligence not to take precautions to avoid such risks. (Lord Oaksey at 863)⁵⁰

p. 236 ↵ Thus, in *Richards v London Borough of Bromley* [2012], the Court of Appeal held that an injury suffered by a school pupil (who had injured her heel when leaving the school through a pair of swing doors) was not reasonably foreseeable. Though there had been one incident four months earlier, this was only superficially similar to the accident in question. Moreover,

[i]n any event the trivial nature of the earlier incident and the risk which it brought to light, seen in the context of thirty years' safe use of the doors by thousands of children and staff, rendered reasonable both the nature of the remedial action which the school authorities proposed to take and the timescale within which they proposed to do it'. (at [14])

In *Perry v Harris* [2008], a child suffered severe and permanent injuries while using a bouncy castle at her friend's birthday party. Finding for the defendant, the parents who had hired the bouncy castle, Lord Phillips CJ held that 'the standard of care she was required to show was that which a reasonably careful parent would show for her own children' (at [37]). In setting the standard of care, what mattered was not merely whether some harm was foreseeable but also the *severity* of the foreseeable harm:

A reasonable parent could foresee that if children indulged in boisterous behaviour on a bouncy castle, there would be a risk that, sooner or later, one child might collide with another and cause that child some physical injury of a type that can be an incident of some contact sports. We do not consider that it was reasonably foreseeable that such injury would be likely to be serious, let alone as severe as the injury sustained by the claimant. (Lord Phillips CJ at [38])

This was followed in the similar case of *Cockbill v Riley* [2013] in which the claimant fractured his spine resulting in incomplete tetraplegia after entering a paddling pool (it was not clear how he had done so) while at a party to celebrate the end of his GCSEs. The court held that it was *not* reasonably foreseeable that someone would attempt to carry out a dive or a belly-flop and thus suffer grave injury and so the defendant was not in breach of his duty of care to the claimant.

It is also clear from the cases that the conduct of the defendant is assessed at the time of the alleged breach. It is a test of foresight, not hindsight (*Roe v Ministry of Health* [1954]). If something seems to be acceptable at the time—that is, if the risk of injury is low—then it is unlikely to be considered negligent.

***Roe v Ministry of Health* [1954] 2 QB 66 (CA)**

The claimants were paralysed after being injected with contaminated nupercaine (a spinal anaesthetic) during a routine, minor operation. The nupercaine had been contained in sealed glass ampoules and stored in a solution of phenol prior to use. Unknown to the hospital, the phenol had percolated into the ampoules through invisible cracks or molecular flaws, causing the claimants' paralysis.

Rejecting the claims, the Court of Appeal held that, though it was clear in hindsight that the hospital was at fault, *at the time of the operation* neither the anaesthetist nor any of the hospital staff knew of the dangers of storing glass ampoules in the phenol solution. The test applied was the standard of medical knowledge when the accident occurred in 1947: ‘We must not look at the 1947 accident with 1954 spectacles’ (Denning LJ at 84).

- p. 237 ↩ The approach in *Roe* was affirmed in *Williams v University of Birmingham* [2011]. The widow of a former physics undergraduate, who had conducted experiments in an asbestos-lagged service tunnel under the science department at the University of Birmingham and who had since died of malignant mesothelioma in his left lung, brought a claim against the university in negligence. The university admitted that the claimant’s husband would have been exposed to asbestos as a student, but argued that the exposure was too small to ground a claim. Rejecting the claim, Atkins LJ (giving the leading judgment) adapted Denning LJ’s ‘graphic phrase’, to argue that ‘we must not look at what happened in the tunnel in 1974 through 2009 or 2011 spectacles’ (at [38]). The state of the lagging was not such as should have alerted the university to a reasonably foreseeable risk that people using the tunnel might be exposed to an unacceptable risk of an asbestos-related injury (at [60]–[61]).

Pause for reflection

A leading medical textbook published in 1951 (after the accident which formed the basis of the claim in *Roe*) warns medical practitioners to ‘never place ampoules of local anaesthetic solution in alcohol or spirit. This common practice is probably responsible for some cases of permanent paralysis reported after spinal analgesia’ (*Roe* at 86). However, despite this significant societal *benefit*—it was the injuries suffered by the claimants in *Roe* which first alerted the medical profession to this particular danger—the effect of the Court of Appeal’s decision was that the full *cost* or burden of this medical advance was borne solely by the unfortunate men. This apparent injustice was rejected by Denning LJ:

[W]e should be doing a disservice to the community at large if we impose liability on hospitals and doctors for everything that happens to go wrong ... Initiatives would be stifled and confidence shaken ... We must insist on due care for the patient at every point, but we must not condemn as negligence that which is only a misadventure. (at 87)

Do you agree? Compare Denning LJ’s reasoning in *Roe* with that in *Nettleship*—why do you think he adopted such a markedly different approach in this case?

Check our pointers <https://iws.oup.support.com/ebook/access/content/horsey9e-student-resources/horsey9e-chapter-8-pointers-to-pause-for-reflection-and-counterpoint-boxes?options=showName> on how to answer the questions posed in this 'Pause for reflection'.

8.4.2 Seriousness of the injury

p. 238 The second factor the courts take into account is the seriousness of the injury should it occur. Generally, the more serious the potential injury, the more likely the defendant will be found to have fallen below the required standard of care should it materialise. After all, 'the law expects of a man a great deal more care in carrying a pound of dynamite than a pound of butter' (Singleton LJ, quoting Percy H Winfield, in *Beckett v Newalls Insulation Co Ltd* [1953] at 15). In *Paris v Stepney Borough Council* [1951] the claimant, a garage hand, suffered serious injury when a metal chip flew into one of his eyes while he was working. Unfortunately, he was already (as his employers knew) blind in his other eye. The injury, therefore, left him effectively blind. He sued his employers, claiming they were negligent in failing to provide safety goggles and to require their use as part of a safe system of work.⁵¹ The majority of the House of Lords agreed. Though the chance of injury was low, the seriousness of the consequences should an accident occur must also be taken into account when assessing the precautions a reasonable employer should take to ensure the safety of their workforce. The provision of safety goggles was 'obviously necessary when a one-eyed man was put to [this] kind of work' (at 383).

Similarly, in *Overseas Tankship (UK) Ltd v Miller Steamship Co Pty Ltd (The Wagon Mound) (No 2)* [1967] the defendants carelessly transferred furnace oil from a nearby wharf onto a vessel, the *Wagon Mound*, which was moored in Sydney Harbour, causing a large quantity of the oil to spill into the harbour where it accumulated around the wharf and the claimants' vessels. The owners of the wharf were carrying out repairs on the claimants' ships which caused pieces of hot metal to fly off and into the wharf. On one such occasion it is thought that the metal fell onto an object supporting a piece of inflammable material in the oil-covered water, which subsequently ignited. This caused the oil to catch alight and the ensuing fire quickly destroyed the wharf and the claimants' vessels. The Privy Council held that although the chance of the oil catching fire was very low it was nevertheless a real one and, given the very grave consequences should the risk materialise, there was no justification for the defendant's failure to take steps to eliminate it given how easy it was for them to prevent the spillage (had they taken reasonable care in the first place when filling up the vessel). On this basis, the defendants had fallen below the standard of care required—the reasonable man would have taken precautions.

If a real risk is one which would occur to the mind of a reasonable man in the position of the defendant's servant and which he would not brush aside as far-fetched ... then surely he would not neglect such a risk if action to eliminate it presented no difficulty, involved no disadvantage, and required no expense. (at 643–4)⁵²

8.4.3 Cost of taking precautions

The courts also consider the cost to the defendant of taking precautions against the risk. The lower the cost—whether in terms of time or money—the more reasonable it is that someone should take precautions. Thus, it is likely (all things being equal) that the courts would consider a driver who fails to slow down while driving near a school at the end of the school day to have fallen below the standard of care required of a reasonable driver—the cost to the driver of driving more slowly is unlikely to be seen as unreasonably high. If, on the other hand, the cost of taking precautions is very onerous, it is less likely that it will be considered reasonable for the defendants to bear such costs—especially, but not only, when the risk of injury is low (*Latimer v AEC Ltd* [1953]).⁵³

p. 239 Traditionally the courts have been reluctant to interfere with the budgetary decisions made in relation to limited resources by public authorities. In *Knight v Home Office* [1990], a mentally ill prisoner committed suicide. As a known suicide risk, he had been observed every 15 minutes. The court recognised that while lack of funds could never be a complete defence to insufficient safety precautions, the facilities of a prison hospital were necessarily different to a specialist psychiatric hospital and so, on the facts, there was no breach of the prison officers' duty of care. However, in *Goldscheider v Royal Opera House Covent Garden Foundation* [2019] the Court of Appeal found the Royal Opera House (ROH), a charity based in Covent Garden, London, liable for the injuries suffered by one of its performers. The claimant, a viola player in the orchestra of the ROH, was able to recover for the damage caused to his hearing as a result by being placed too close to the trumpet section in the orchestra 'pit'. Despite undertaking a risk assessment and providing performers with ear plugs and training, the ROH had failed to take 'all reasonably practical steps' to protect the claimant's hearing in breach of the Control of Noise at Work Regulations 2005. McCombe and Bean LJ continued:

It is in our judgment particularly significant that the pit was [later] reconfigured ... with the brass instruments being split up. There is no evidence that this caused an unacceptable reduction (or indeed any reduction at all) in the artistic standards of ... [the Wagner opera the claimant was performing] when it came to be performed in public. Alterations made by defendants after a workplace accident do not necessarily demonstrate liability retrospectively, but they do make it very difficult for the defendant to prove that all reasonably practicable steps had already been taken. (at [42])

8.4.4 Social value of the activity

While an assessment of the 'cost of taking precautions' goes to the *private* costs of adhering to a particular standard of care imposed on the individual, this final factor relates to the *social* costs of the activity. The greater the social value of the activity, the more likely the courts will find it reasonable to have dispensed with safety precautions.

While there is no special exemption for any of the emergency services from the law of negligence (the question remains whether the defendant has behaved as a reasonable man), the court will take into account the emergency in which the defendant is acting. The standard of care will usually therefore be lower where the defendant is acting in the 'heat of the moment' or in an emergency or rescue situation.

***Watt v Hertfordshire County Council* [1954] 1 WLR 835 (CA)**

A fireman was seriously injured by heavy lifting-gear while travelling in the back of a lorry on the way to an accident where a woman was trapped under a vehicle. The lorry had not been specially fitted to carry the gear in an emergency.

The Court of Appeal, rejecting the fireman's claim, held that the public benefit justified taking the risks associated with failing to adequately secure the lifting-gear in the back of the lorry:

It is well settled that in measuring due care you must balance the risk against the measures necessary to eliminate the risk. To that proposition there ought to be added this: you must balance the risk against the end to be achieved ... The saving of life or limb justifies taking considerable risk. (Denning LJ at 838)

If, therefore, the same accident had occurred in pursuit of a commercial end then the claimant would have recovered: 'the commercial end to make a profit is very different from the human end to save life or limb' (at 838).⁵⁴

p. 240 ↩ But what about other 'socially valuable' activities such as sports events or games between children?

In the context of sporting events,

the threshold for liability is in practice inevitably high; the proof of a breach of duty will not flow from proof of no more than an error of judgment or from mere proof of a momentary lapse in skill (and thus care) respectively when subject to the stresses of a race [or game or match]. Such are no more than incidents inherent in the nature of the sport. (HHJ Walden-Smith, *Tylicki v Gibbons* [2021] at [26] referring to *Holland J in Caldwell*)

Liability is 'fact specific' and 'cannot be resolved in a vacuum' (*Caldwell v Maquire* [2002] at [30]). In *Condon v Basi* [1985], the first case to consider the standard of care relating to players in the context of competitive sports, the claimant was injured by a 'reckless and dangerous' sliding tackle during a Sunday league football game. The trial judge held that the tackle amounted to 'serious and dangerous foul play which showed a reckless disregard of the Plaintiff's safety and which fell far below the standards which might reasonably be expected in anyone pursuing the game'. Upholding this decision, Sir John Donaldson MR confirmed that the objective standard of care was to be assessed in context: 'there will of course be a higher degree of care required of a player in a First Division football match than of a player in a Fourth Division football match'. This was followed in *Fulham Football Club v Jones* [2022]. The claimant suffered a career-ending injury as the result of being tackled by the defendant during an under-18 football match. Finding for the defendant, Lane J held that in determining the relevant standard of care in a negligence claim in the context of sports injuries

the court will have regard to the context and realities of the relevant sport and that the standard of care for establishing liability in negligence is ‘materially higher’ than a mere breach of the rules of the game (the referee in this case had not awarded a foul and allowed play to continue).⁵⁵

This approach has also been adopted outside the context of organised sports. In *Blake v Galloway* [2004], for example, a teenager was injured while playing a ‘game’ which involved throwing bark and twigs at each other. The Court of Appeal held that there had been no breach of duty:

If the defendant in the present case had departed from the tacit understandings or conventions of the play and, for example, had thrown a stone at the claimant, or deliberately aimed the piece of bark at the claimant’s head, then there might have been a breach of the duty of care. But what happened here was, at its highest, ‘an error of judgment or lapse of skill’ (to quote from Diplock LJ), and that is not sufficient to amount to a failure to take reasonable care in the circumstances of horseplay such as that in which these youths were engaged ... This was a most unfortunate accident, but it was just that. Young persons will always want to play vigorous games and indulge in horseplay, and from time to time accidents will occur and injuries will be caused. But, broadly speaking, the victims of such accidents will usually not be able to recover damages unless they can show that the injury has been caused by a failure to take care which amounts to recklessness or a very high degree of carelessness, or that it was caused deliberately (i.e. with intent to cause harm). (Dyson LJ at [15], [25])

However, it is clear following *Scout Association v Barnes* [2010] that the social value of an activity *as a whole* does not mean that *all* examples of that activity are acceptable, whatever the risk:

Of course, the law of tort must not interfere with activities just because they carry some risk. Of course, the law of tort must not stamp out socially desirable activities. But whether the social
↪ benefit of an activity is such that the degree of risk it entails is acceptable is a question of fact, degree and judgment, which must be decided on an individual basis and not by a broad brush approach. (Smith LJ at [49])

In *Barnes*, the claimant was injured while playing a game called ‘Objects in the Dark’ which involved running around a pile of wooden blocks in a scout hall and attempting to grab a block once the lights were switched off (with only minimal lighting provided by emergency exit lights). The majority of the Court of Appeal upheld the trial judge’s decision that the defendant had breached its duty—the value of playing the game in the dark as a way of adding ‘spice’ to increase the boys’ enjoyment of the game, and which ‘might well encourage boys to attend scouts—a desirable objective—did not justify the increased foreseeable risk’ (Smith LJ at 46).

Pause for reflection

Though it is clear from the majority judgments in *Barnes* that the trial judge had appropriately weighed the costs and benefits of the competing factors at issue in the case, there is considerable force in Ward LJ's dissenting opening paragraph:

I have to confess that I have found it uncommonly difficult to reach a confident judgment in this case. Here was a big strong thirteen year old lad, well-used to rough and tumble, playing rugby with distinction for his county, ever ready to take the bumps and the bruises, ever willingly to put his body on the line for the thrill of his sport. For him, you get hurt, you get up, and you get on with it. He brought the same enthusiasm and competitive instincts to his participation in his local Scout troop. He was the least likely boy to need wrapping in cotton wool. So, is awarding him damages for an injury suffered playing the game, 'Objects in the Dark', not an example of an overprotective nanny state robbing youth of fun simply because there was some risk involved in the exercise? Is this a decision which emasculates those responsible for caring for our children and in so doing, enfeebles the children themselves? Where do you draw the line? I have found that hard to answer. (at [50])

Do you think that the majority drew the line correctly in this case? To what extent, if at all, can it be reconciled with the decision in *Blake v Galloway*?

Test your understanding by trying this section's self-test questions <https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-self-test-questions-8-4>.

8.5 Parliamentary interventions

In recent years, successive governments have made various (unnecessary) interventions in an effort to address the *perception* of a growing compensation culture in the UK.⁵⁶ The Compensation Act 2006 is one example. Speaking in 2005, the then Prime Minister, Tony Blair, suggested its purpose was to:

clarify the existing common law on negligence to make clear that there is no liability in negligence for untoward incidents that could not be avoided by taking reasonable care or exercising
↪ reasonable skill ... This will send a strong signal and it will also reduce risk-averse behaviour by providing reassurance to those who may be concerned about possible litigation, such as volunteers, teachers and local authorities.⁵⁷

p. 242

To this end, section 1 determines that the courts when considering a claim in negligence or breach of statutory duty *may* ‘have regard to’ the wider impact of their assessment of the appropriate standard of care in a particular case—in particular, whether a requirement to take those steps might: (a) prevent a desirable activity from being undertaken at all, to a particular extent or in a particular way; or (b) discourage persons from undertaking functions in connection with a desirable activity. Of course, this adds little to our understanding of the tort of negligence. As Jackson LJ in *Barnes* notes, it simply restates what we already know, that:

the function of the law of tort [is] to deter negligent conduct and to compensate those who are the victims of such conduct. It is not the function of the law of tort to eliminate every iota of risk or to stamp out socially desirable activities. (at [34])⁵⁸

Nevertheless, in 2015 the Compensation Act was joined by another unnecessary piece of legislation: the Social Action, Responsibility and Heroism (SARAH) Act 2015. At just 176 words long and enacted as part of the Coalition government’s ‘wider programme to encourage participation in civil society and ... a specific commitment to “take a range of measures to encourage volunteering and involvement in social action”’,⁵⁹ the SARAH Act lists three matters—social action, responsibility and heroism—to which a court must have regard in determining a claim in negligence or breach of statutory duty. Save for a passing reference in the unrelated case of *Long v Elegant Resorts Ltd* [2021] it is yet to be applied in a reported case.

Watch our video ‘Where Angels Fear to Tread: Unnecessarily law-making and the Social Action, Responsibility and Heroism Act 2015’ where we look at what the 2015 Act tells us about what happens when Parliament steps in to ‘fix’ a non-existent problem.

Video playback is not supported in this format.

Video 8: Where angels fear to tread: unnecessary law-making and the Social Action, Responsibility and Heroism Act 2015

Test your understanding by trying this section’s self-test questions. <<https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-self-test-questions-8-5>>.

8.6 Establishing breach

You will remember that in order to establish whether the defendant has breached their duty of care two questions need to be considered:

- (1) How the defendant *ought* to have behaved in the circumstances—what was the required standard of care in these circumstances?

(2) How the defendant *did* behave—did they fall below the standard of care required?

So far, this chapter has concentrated solely on the first of these questions. The second question—that is, proof of negligence—is largely a question of fact to be determined by the court on the evidence before them. The burden is on the claimant to establish, on the balance of probabilities—that is, that it is more likely than not—that the defendant’s actions fell below the required standard of care.⁶⁰

p. 243 ↵ At times this can be extremely difficult. In many cases, the claimant will have to rely on the defendant releasing certain information to them (which they are unlikely, understandably, to be willing to do) or the information may be highly technical or simply very dense. The latter is particularly so in relation to litigation arising out of a defective drug where it can be very time-consuming and expensive to sift through the scientific and medical evidence.⁶¹ Things are slightly more straightforward in relation to road traffic accidents. Section 11 of the Civil Evidence Act 1968 shifts the burden of proof onto the defendant to show that he was *not* negligent and that his conduct (although criminal) does not amount to civil negligence. While a criminal conviction (or breach of the Highway Code (*Brown v Paterson* [2010])) does not automatically establish negligence, it is admissible in civil proceedings as *prima facie* evidence of negligence. It is highly likely, therefore, that a driver who, for example, is convicted of injuring another by their careless driving, will also be found to be negligent.⁶²

The courts may also infer negligence from the circumstances in which the accident or injury took place. This process is usually described by the phrase *res ipsa loquitur* meaning ‘the thing speaks for itself’. Academics disagree on the precise status of the ‘doctrine’.⁶³ Some argue that it simply reflects the common-sense view that sometimes the likelihood that the accident was caused by the defendant’s negligence is such that it is not necessary to explain it, while others suggest that it is a more formal doctrine which, in certain circumstances, shifts the burden of proof from the claimant to the defendant (meaning that they will be liable if they are unable to prove, on the balance of probabilities, the *absence* of fault on their part). Most agree that this latter view goes too far. We prefer the view of Jenny Steele, who describes *res ipsa loquitur* as not so much a doctrine ‘as a complex name for a common-sense inference from the facts’.⁶⁴ So understood, *res ipsa loquitur* does *not* shift or reverse the burden of proof onto the defendant. It simply means that sometimes the circumstances of the negligence can be evidence of carelessness. As such it is a welcome and effective means of redressing the balance of power between claimant and defendant where evidential difficulties make it hard for the claimant to establish negligence.

Whatever its status, the ‘doctrine’ stems from the judgment of Erle CJ in *Scott v London and St Katherine Docks Co* [1895] in which the claimant was injured by falling bags of sugar which were being loaded by the defendant’s crane. In response to the defendant’s failure to explain how the injury occurred, Erle CJ stated:

There must be reasonable evidence of negligence, but, where the thing is shown to be under the management of the defendant, or his servants, and the accident is such as, in the ordinary course of things, does not happen if those who have the management of the machinery use proper care, it affords reasonable evidence, in the absence of explanation of the defendant, that the accident arose from want of care. (at 601)

Two things need to be met in order for the rule to come into play (although these often run together). First, the thing which caused the accident needs to be ‘under the management of the defendant, or his servants’, that is, the defendant needs to have control of the thing that caused the injury. A defendant will not be liable if, for example, someone steals their car and causes an accident, or for a hotel guest throwing furniture out of a hotel window (*Larson v St Francis Hotel* [1948]). Secondly, the accident must be such as ‘in the ordinary course of things, does not happen if those who have the management of the [things] use proper care’. This, largely, depends on the circumstances of the case—it will be easier to prove negligence if you slip on a wet floor near a swimming pool or gym (where the occupiers may be expected to know that the floor might be wet and take appropriate precautions) than in a shopping centre (*Ward v Tesco Stores Ltd* [1976]). The question for the court is whether the evidence taken as a whole points in the direction of the defendant’s negligence. Thus, where the defendant is unable to explain how the accident occurred but is able to show that they exercised all reasonable care in the circumstances, they will not be found liable (*J v North Lincolnshire County Council* [2000]).

Test your understanding by trying this section’s self-test questions <<https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-self-test-questions-8-6>>.

8.7 Conclusion

In this chapter, we have considered the second part of a claim in the tort of negligence—*breach of duty of care*. This occurs where a defendant has fallen below the standard of care required by law. In setting the standard of care, the courts seek to balance issues relating to the public benefit against individual freedom and responsibility. This takes the form of two questions: (1) what was the required standard of care in these circumstances? (2) And did the defendant fall below this standard? When setting the standard of care the courts compare the actions of the defendant to those of the ‘reasonable man’. This is an objective test (although the courts, on occasions, may incorporate the defendant’s age and skills into their assessment). When setting the standard of care required of the defendant—that is, what the defendant *ought* to have done or what is *reasonable* in the circumstances of the case—the courts take a number of factors into account: probability or risk of the injury; the seriousness of the injury; the cost of taking precautions; and sometimes the social value of the activity.

The second question—that is, proof of negligence—is largely a question of fact to be determined by the court on the evidence before them. The claimant is sometimes helped when seeking to establish on the balance of probabilities that the defendant breached their duty of care by the ‘doctrine’ of *res ipsa loquitur*.

End-of-chapter questions

After reading the chapter carefully, try answering the questions which follow.

- 1 In **Bolton v Stone** the cricket club was not held liable. What was the role of reasonable foreseeability? Do you agree with the outcome of the case?
- 2 What was the reasoning behind **Nettleship**? Is it a fair decision? Fair (or unfair) to whom?
- 3 When should a doctor warn patients of a risk related to their treatment?
- 4 'It is better to control liability by adjusting the standard of care than by restricting the circumstances in which a duty of care arises.' Discuss.

If you would like to know what we think, check the **answer guidance** <https://iws.oup.support.com/ebook/access/content/horsey9e-student-resources/horsey9e-chapter-8-answers-to-end-of-chapter-questions?options=showName>.

p. 245

Answering the problem question

Consider again the problem question at the start of this chapter. Now having read about the topic, **what would be your advice to the various parties?**

Here are some pointers to get you started:

- Mpho's claim against Gabriel is relatively straightforward—the question is whether Gabriel has fallen below the standard expected of a reasonable driver. You should consider what difference, if any, the fact that he's just passed his test makes.
- Aput's claim against the doctor is more complicated. You will need to consider whether the doctor has breached their duty of care in failing to warn them about the risks of surgery—has Aput been given the opportunity to give their 'informed consent'? See **Bolam**, **Montgomery** and **McCulloch**.
- Also remember the question asks you to 'advise Mpho and Aput' so although this is a 'breach' question you should consider all the elements of a claim in negligence.

If you need some more guidance:

- See the **annotated version of the problem with issues and cases to consider** <https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-chapter-8-annotated-problem-question-interactive>.
- Check your answer against the **suggested outline answer** <https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-chapter-8-answers-to-problem-question>.

Further reading

Much of the academic writing in this area focuses on medical negligence and, in particular, the effect of **Bolam** and more recently **Montgomery**.

Abraham, Haim 'Queering the Reasonable Person' in Kirsty Horsey (ed) *Diverse Voices in Tort Law* (Bristol University Press 2023) 57

Beever, Allan 'Negligence and Utility' (2017) 17 Oxford University Commonwealth Law Journal 85

Brazier, Margaret and José Miola 'Bye-Bye *Bolam*: A Medical Litigation Revolution?' (2000) 8 Medical Law Review 85

Cave, Emma and Caterina Milo 'Informing Patient: The *Bolam* Legacy' (2020) 20 Medical Law International 103

Conaghan, Joanne 'Tort Law and the Feminist Critique of Reason' in Anne Bottomley (ed) *Feminist Perspectives on the Foundational Subjects of Law* (Cavendish 1996) 47

Gardiner, Bruce 'Liability for Sporting Injuries' [2008] Journal of Personal Injury Law 16

Goudkamp, James and Melody Ihuoma 'A Tour of the Tort of Negligence' [2016] Professional Negligence 137

Herbert, Rebecca 'The Compensation Act 2006' [2006] Journal of Personal Injury Law 337

p. 246 Heywood, Robert and José Miola 'The Changing Face of Pre-operative Medical Disclosure: Placing the Patient at the Heart of the Matter' (2017) 133 Law Quarterly Review 296 ↵

Miola, José 'The Standard of Care in Medical Negligence—Still Reasonably Troublesome?' in Janice Richardson and Erika Rackley (eds) *Feminist Perspectives on Tort Law* (Routledge 2012) 163

Mulheron, Rachael 'Trumping *Bolam*: A Critical Legal Analysis of *Bolitho*'s "Gloss"' [2010] Cambridge Law Journal 609

Nolan, Donal 'Varying the Standard of Care in Negligence' [2013] Cambridge Law Journal 651

Sheldon, Sally 'A Responsible Body of Medical Men Skilled in that Particular Art: Rethinking the *Bolam* Test' in Sally Sheldon and Michael Thomson (eds) *Feminist Perspectives on Health Care Law* (Cavendish 1998)

Additional suggestions for further reading can be found in this chapter's **web links** <<https://iws.oup.support.com/ebook/access/content/horsey9e-student-resources/horsey9e-chapter-8-web-links?options=showName>>.

Notes

¹ Since July 2021, Little Amal, a 12-foot puppet, has travelled over 9,000km across 13 countries representing all children fleeing war, violence and persecution. Read more here: www.walkwithamal.org <<http://www.walkwithamal.org>>.

² Tony Weir *An Introduction to Tort Law* (2nd edn OUP 2006) 57.

³ See e.g. Press Association ‘Paralysed teacher loses £5m “welly-wanging” damages claim’ *The Guardian* 13 August 2012 (discussing *Cornish Glennroy Blair-Ford v CRS Adventures Ltd* [2012]), BBC News ‘Charlotte Shaw mother loses death claim’ 28 June 2012 (discussing *Wilkin-Shaw v Fuller & Kingsley School Bideford Enterprises Ltd* [2012]) and BBC News ‘Former RAF man wins right to sue MoD over game injury’ 2 February 2011 (discussing *Uren v Corporate Leisure (UK) Ltd* [2011]); Alexandra Topping ‘Parents win appeal over head injury on bouncy castle’ *The Guardian* 1 August 2008 (discussing *Perry v Harris* [2008]).

⁴ See e.g. Ragnar E Löfstedt ‘Reclaiming Health and Safety for all: a review of progress one year on’ January 2013; Ragnar E Löfstedt ‘Reclaiming Health and Safety for all: an independent review of health and safety legislation’ November 2011; Health and Safety Executive ‘School trips and outdoor learning activities: tackling the health and safety myths’ 6 June 2011.

⁵ Indeed school compensation claims are increasingly ‘big business’—see e.g. Matthew Davis ‘Schools pay £1.5m in compensation cases including £7,500 for a child who had a splinter’ *Daily Mirror* 22 August 2015. One may have a certain amount of sympathy with Tomlinson LJ’s observation in *Richards v London Borough of Bromley* [2012] that ‘[i]t needs to be understood that not every misfortune occurring on school premises attracts compensation’ (at [15]). See further on the ‘compensation culture’, **section 1.3.2**.

⁶ Joanne Conaghan ‘Tort Law and the Feminist Critique of Reason’ in Anne Bottomley (ed) *Feminist Perspectives on the Foundational Subjects of Law* (Cavendish 1996) 47, 52.

⁷ *ibid* 58. See also Mayo Moran *Rethinking the Reasonable Person* (OUP 2003) and Haim Abraham ‘Queering the Reasonable Person’ in Kirsty Horsey (ed) *Diverse Voices in Tort Law* (Bristol University Press 2024) 57.

⁸ Not everyone agrees with this point. See e.g. Peter Cane and James Goudkamp who argue that the reasonable person is invoked in order to obscure the role of the judge as a policy-maker (*Atiyah’s Accidents, Compensation and the Law* (9th edn CUP 2018) 30).

⁹ However, as we will see later, the courts have developed a number of modifications to the general standard, e.g. in relation to children and people professing to have a particular skill.

¹⁰ On the relevance of a criminal conviction on a civil claim, see **section 8.6**.

¹¹ See **section 10.4**.

¹² Cane and Goudkamp **n8** 40.

¹³ The Australian High Court, in a case on very similar facts, refused to follow *Nettleship* and described it as ‘contrary to common sense and the concept of what is reasonable in the circumstances’ (*Cook v Cook* [1986]) (though in this case the defendant had fallen below the (lower) standard of care required of a learner driver).

¹⁴ Though cf *Wooldridge v George* [2017] in which it was held that the driver, who was within the speed limit and who was not distracted, should have seen the claimant crossing the road and taken evasive action to avoid hitting him.

¹⁵ An argument made by Patrick Atiyah in *The Damages Lottery* (Hart 1997).

¹⁶ *Tort Law* (Macmillan 2021) 5.

¹⁷ See further James Goudkamp and Melody Ihuoma ‘A Tour of the Tort of Negligence’ [2016] *Professional Negligence* 137, 138–44 and Maria Orchard ‘Liability in Negligence of the Mentally Ill: A Comment on *Dunnage v Randell*’ (2016) 45 *Common Law World Review* 366.

- ¹⁸ WE Peel and James Goudkamp *Winfield & Jolowicz on Tort* (19th edn Sweet & Maxwell 2014) 277.
- ¹⁹ See also e.g. *Wells v Cooper* [1958], where a do-it-yourself (DIY) carpenter was not liable for injuries caused by his failure to use adequate screws to fix a door handle. He had not fallen below the standard of care expected of a reasonably competent non-professional carpenter (which was lower than that required of a professional carpenter). See also *Perry v Harris* [2008] (discussed in **section 8.4.1**).
- ²⁰ Note: the decision in *Wilsher* was appealed to the House of Lords on the issue of causation where the standard of care issue was not discussed.
- ²¹ Margaret Brazier and José Miola ‘Bye-Bye *Bolam*: A Medical Litigation Revolution?’ (2000) 8 Medical Law Review 85, 87.
- ²² The issue as to the extent to which ‘doctors know best’ was alive and kicking in the heated debates surrounding the (failed) Medical Innovation Bill 2014–15 (the so-called ‘Saatchi Bill’) and its later reincarnation as the Access to Medical Treatments (Innovation) Act 2016. They both attracted significant—and well-judged—criticism from legal and medical professionals alike. See e.g. José Miola ‘Bye-Bye *Bolitho*? The Curious Case of the Medical Innovation Bill’ (2015) 15 Medical Law International 124.
- ²³ Sally Sheldon ‘A Responsible Body of Medical Men Skilled in that Particular Art: Rethinking the *Bolam* Test’ in Sally Sheldon and Michael Thomson (eds) *Feminist Perspectives on Health Care Law* (Cavendish 1998) 20–1.
- ²⁴ *ibid* 19. For an insightful ‘update’ of Sheldon’s critique of the standard of care, see José Miola ‘The Standard of Care in Medical Negligence—Still Reasonably Troublesome?’ in Janice Richardson and Erika Rackley (eds) *Feminist Perspectives on Tort Law* (Routledge 2012) 163.
- ²⁵ Lord Woolf ‘Are the Courts Excessively Deferential to the Medical Profession?’ (2001) 9 Medical Law Review 1–16 at 1.
- ²⁶ Miola **n24** 614. See further *McGuinn v Lewisham and Greenwich NHS Trust* [2017] applying *Bolitho* to find the doctor in breach of their duty of care (in failing to appreciate that the fetus the claimant was carrying was at risk of suffering from microcephaly) because their actions ‘lacked a sufficiently logical and rational basis’ (at [175]) and Simon Fox ‘*Bolam* is Dead. Long Live *Bolam*’ (2019) 4 Professional Negligence 213.
- ²⁷ Rachael Mulheron ‘Trumping *Bolam*: A Critical Legal Analysis of *Bolitho*’s “Gloss”’ [2010] Cambridge Law Journal 609, 613.
- ²⁸ Indeed, in *Ministry of Justice v Carter* [2010] the Court of Appeal overturned the decision of a trial judge who did this. For a clear example of the judicial process of determining whether a defendant has breached their duty of care, see *Wells and Smith v University Hospital Southampton NHS Foundation Trust* [2015].
- ²⁹ Brazier and Miola **n21**.
- ³⁰ Lords Reed and Kerr, *Montgomery* at [86]. In *Chester*, the majority of the House of Lords stepped around *Sidaway* (rather than deviating from it) to hold that a doctor who had not informed the patient of a very small risk which accompanied the course of treatment he was suggesting had not only breached his duty of care but also that the patient’s ability to make an informed choice about her treatment was so important that they were willing to extend the rules on causation in order to allow her claim: ‘Her right of autonomy and dignity can and ought to be vindicated by a narrow and modest departure from traditional causation principles’ (Lord Steyn at [24]). On the causation aspects of *Chester*, see further **section 9.2.2.3**.

- ³¹ It took 16 years for **Montgomery** to make its way through the courts. At its heart is the care of a—now—16-year-old boy. It has since been reported that Ms Montgomery has been awarded between £5–5.25 million in damages which will allow her, in her words ‘to ensure Sam receives the best possible care for the rest of his life’ (BBC News ‘Nadine Montgomery wins £5m from NHS Lanarkshire over brain damage to son’ 11 March 2015). You may also like to listen to this excellent podcast episode (Kings Chambers Debrief E10: ‘*Montgomery v Lanarkshire Health Board* (Part II <http://www.buzzsprout.com/179951/2134727>)’) produced by Kings Chambers which features a conversation between Nigel Poole KC and Nadine Montgomery. They talk about her experience of taking her case all the way to the Supreme Court and the ramifications of the judgment. The podcast is just over 22 minutes long so if you don’t have time to listen to it right now, just make a note to return to it later—or perhaps download it to listen to on the way to your tort seminar!
- ³² See Kirsty Keywood ‘The Death of *Sidaway*: Values, Judgments and Informed Consent’ *Journal of Medical Ethics blog* 15 March 2015.
- ³³ See discussion of the so-called forced caesarean section cases in **section 15.5.1**.
- ³⁴ See further Erika Rackley *Women Judging and the Judiciary: From Difference to Diversity* (Routledge 2013) chs 5 and 6.
- ³⁵ Keywood **n32**.
- ³⁶ NHS Litigation Authority ‘Ten Years of Maternity Claims: An Analysis of NHS Litigation Authority Data’ (2012).
- ³⁷ See here the Royal College of Surgeons guidance on **Montgomery** discussed in David Hart ‘Informed consent: Surgeons respond to *Montgomery*’ *UK Human Rights blog* 30 October 2016.
- ³⁸ See David Hart ‘Supreme Court reverses informed consent ruling: *Sidaway* is dead’ *UK Human Rights blog* 13 March 2015.
- ³⁹ Andrew Hockton ‘“Informed consent” has arrived’ *The Times* 19 March 2015.
- ⁴⁰ See further, Nigel Poole ‘After *Montgomery*—Part Two: Recommended Treatment, Risk and Dialogue’ *Learned Friend* 12 September 2018.
- ⁴¹ NHS Resolution ‘Case of note: *McCulloch (deceased) v Forth Valley Health Board* (Supreme Court—12 July 2023)’ 19 October 2023.
- ⁴² See further Louise Austin ‘*McCulloch v Forth Valley Health Board* [2023] UKSC 26: Hello *Bolam*, the court’s old friend’ [2024] *Medical Law Review* 264, 273.
- ⁴³ See e.g. Emma Cave and Caterina Milo ‘Informing Patients: The *Bolam* Legacy’ (2020) 20 *Medical Law International* 103; Gemma Turton ‘Informed Consent to Medical Treatment Post-*Montgomery*: Causation and Coincidence’ [2018] *Medical Law Review* 108; George G Buttigieg ‘Re-visiting *Bolam* and *Bolitho* in the Light of *Montgomery v Lanarkshire Health Board*’ (2018) 86 *Medico-Legal Journal* 42; Nigel Poole ‘After *Montgomery*—Part Two: *Bolam* Returns’ *Learned Friend* 12 September 2018; Sarah W Chan et al ‘*Montgomery* and Informed Consent: Where Are We Now?’ (2017) 357 *British Medical Journal* 2224.
- ⁴⁴ For a summary of the literature, see Isabelle Le Gallez et al ‘*Montgomery*’s Legal and Practical Impact: A Systematic Review at 6 Years’ (2022) 28(4) *Journal of Evaluation in Clinical Practice* 690.
- ⁴⁵ For a recent application of this principle, see *Mohmed v Barnes and EUI Ltd* [2019] in which the court held the defendant had not fallen below the standard of a reasonable driver placed in the threatening and rapidly deteriorating situation he found himself in (at [52]).

- ^{46.} ‘Social costs’ here include not only financial but also ‘immeasurable “soft” values such as community concepts of justice, health, life and freedom of conduct’ (*Western Suburbs Hospital v Currie* [1987]). The assessment of community values can be tricky. How are we to compare, to use Cane and Goudkamp’s example, the social value of playing cricket with the safety of passers-by so as to decide whether it is negligent to play cricket without a fence (n8 33)?
- ^{47.} Cane and Goudkamp n8 32.
- ^{48.} Joanne Conaghan and Wade Mansell *The Wrongs of Tort* (2nd edn Pluto Press 1999) 62.
- ^{49.} It was agreed that nuisance could not be established in the absence of negligence (at 860).
- ^{50.} See also *Lewis v Wandsworth London Borough Council* [2020].
- ^{51.} See further **section 13.2.3**.
- ^{52.} Note this is the second of the *Wagon Mound* cases. The first, ***Overseas Tankship (UK) Ltd v Morts Dock and Engineering Co (The Wagon Mound) (No 1)* [1961]**, brought by the owners of the wharf, is discussed in the context of remoteness in **Chapter 9**.
- ^{53.} Discussed in **section 13.2.4**.
- ^{54.} See more recently *MacLeod v The Commissioner of the Police for the Metropolis* [2015] discussed in Karen O’Sullivan ‘Sound the Alarm’ (2015) 165 *New Law Journal* 7678 at 10.
- ^{55.} See further *Vowles v Evans* [2003] and *Elbanna v Clark* [2024] (rugby), *Fulham v Jones* [2022] (football), *Watson v British Boxing Board of Control Ltd* [2001] (boxing) and *Tylicki v Gibbons* [2021] (horse racing).
- ^{56.} Part 2 of the Act addresses the aggressive marketing of the claims management industry, which the government stressed was responsible, alongside irresponsible media reporting, for fostering the *perception* of a compensation culture (see generally **section 1.3.2** and Rebecca Herbert ‘The Compensation Act 2006’ [2006] *Journal of Personal Injury Law* 337).
- ^{57.} Tony Blair ‘Common Sense Culture not Compensation Culture’ speech delivered at Institute of Public Policy Research, 26 May 2005.
- ^{58.} See also comment of Smith LJ in *Uren v Corporate Leisure (UK) Ltd* [2011] that ‘it was common ground that [the Act] ... did not add anything to the common law position’ (at [13]).
- ^{59.} Social Action, Responsibility and Heroism Bill, Explanatory Notes at [6].
- ^{60.} The claimant must provide evidence of carelessness if an action in negligence is to disclose a good cause of action. If the claimant is unable to do this, the defendant may apply for summary judgment (which means the claim will fail without further argument).
- ^{61.} This difficulty was addressed by Parliament in Part 1 of the Consumer Protection Act 1987, see **Chapter 12**.
- ^{62.} See Lord Denning MR’s discussion of the claimant’s liability in ***Nettleship*** at 698–9.
- ^{63.} Patrick Atiyah ‘*Res Ipsa Loquitur* in England and Australia’ (1972) 35 *Modern Law Review* 337.
- ^{64.} Jenny Steele *Tort Law: Text, Cases, and Materials* (5th edn OUP 2017) 157.

Related Books

View the Essential Cases in tort law

Related Links

Test yourself: Multiple choice questions with instant feedback <<https://learninglink.oup.com/access/content/brennan-concentrate6e-student-resources/brennan-concentrate6e-diagnostic-test>>

Find This Title

In the OUP print catalogue <<https://global.oup.com/academic/product/9780198925231>>